

Chapter 8 Case-Based Review Questions

Clean Case Question Bank from Supabase Database (with Clinical Cases & Images)

■ Case A: Patient Jeff (Oropharyngeal Pathology & Dysphagia)

Patient Profile & Case History

- **Patient:** Jeff | **Age:** 30 | **Sex:** Male | **Height:** 6'1" | **Weight:** 180 lbs
 - **Vital Signs:** BP 120/80 | Pulse 60 | Respiration 16
 - **Dental History:** Missing teeth due to boxing trauma (without athletic mouthguard), untreated caries, multiple 5 mm periodontal probing depths, and no dental care while living abroad for several years working in hotel management. Recently returned to the U.S.
 - **Chief Complaint:** Six months of painless **dysphagia** (difficulty swallowing) and throat fullness. Initially noticed redness in the posterior mouth; now sees a thick, white plaque in the posterior throat.
 - **Oral Examination:** Large ulcerative, thick white plaque-like exophytic ("**cauliflower-like**") mass involving the posterior oropharynx with broad tissue involvement, completely obscuring identifiable left tonsillar tissue.
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Q1. Given the profile and information about this patient, what do you believe is the primary cause of the irritation in the posterior oral region?



- a. Amount of alcohol use causing irritation
- b. Possibly HIV infection
- c. Possible allergy-related lesions
- d. HPV infection

Correct Answer: D. HPV infection

- **Clinical Rationale:** Human Papillomavirus (HPV) infection is the primary etiology for exophytic, verrucous, or plaque-like lesions in the posterior oropharynx. Histologic biopsy is required for definitive diagnosis.

Q2. What is the most likely concern in this case study?

- a. Injury to the tonsillar area
- b. A severe infection of the tonsil on the right side
- c. Squamous papilloma / Squamous cell carcinoma
- d. Ingestion of very hot liquids

Correct Answer: C. Squamous papilloma / Squamous cell carcinoma

- **Clinical Rationale:** A painless, cauliflower-like mass obscuring tonsillar tissue raises immediate concern for squamous cell papilloma or HPV-associated squamous cell carcinoma.

Q3. What is the best course of action for this patient?



- a. Suggest that he take antibiotics for at least two weeks and schedule a follow-up appointment
- b. Try to remove some of the tissue and submit it for a biopsy
- c. Refer patient to an experienced ENT practice or an oral surgery, oral pathology, or oral medicine practice
- d. Try to begin some oral hygiene procedures to contain the possible infection

Correct Answer: C. Refer patient to an experienced ENT practice or an oral surgery, oral pathology, or oral medicine practice

- **Clinical Rationale:** Large, unexplained oropharyngeal lesions require immediate specialist referral to an Otolaryngologist (ENT) or Oral Pathologist for diagnostic tissue biopsy.

Q4. Higher rates of HPV oral infections are found in which of the following?

- a. Males

- b. Females
- c. Both males and females
- d. None

Correct Answer: A. Males

- **Clinical Rationale:** U.S. population studies (NHANES) indicate oral HPV prevalence is 10–11% in men vs. 3–4% in women, explaining why HPV-associated oropharyngeal cancers occur much more frequently in males.

Q5. The cure rate for HPV oral cancers when found early is:

- a. 50%–55%
- b. 65%–75%
- c. 75%–85%
- d. 90%–95%

Correct Answer: C. 75%–85%

- **Clinical Rationale:** Early detection of HPV-driven oropharyngeal carcinomas carries a favorable 75% to 85% cure/survival rate.

Q6. All of the following are representative of contracting the HPV virus EXCEPT one. Which one is the EXCEPTION?

- a. Skin to skin contact
- b. Food borne contamination
- c. Deep kissing
- d. Sexual contact from oral, genital exposure

Correct Answer: B. Food borne contamination

- **Clinical Rationale:** HPV is transmitted via direct mucosal/skin contact or sexual activity; it is NOT spread via food or beverage contamination.

Q7. Performing a full dental exam with both intraoral palpation and extraoral palpation is crucial for early detection of an HPV infection. Why is palpation of the cervical chain of lymph nodes and salivary gland tissue so important?

- a. The lymph nodes are always enlarged in a disease process
- b. The salivary glands produce more saliva with oral diseases
- c. HPV has an affinity for the salivary gland ducts
- d. The patient may complain of pain in the cervical area of the neck

Correct Answer: C. HPV has an affinity for the salivary gland ducts

- **Clinical Rationale:** HPV exhibits an affinity for ductal epithelium, making cervical lymph node and salivary duct palpation essential during screening.

Q8. Dysphonia describes which of the following?

- a. A lack of taste
- b. Painful swallowing
- c. Pain with nerve stimulation
- d. Hoarseness of the voice

Correct Answer: D. Hoarseness of the voice

- **Clinical Rationale:** Dysphonia is the clinical term for hoarseness or an abnormal voice quality.

■ Case B: Lower Lip Papule & Salivary Pathology (Mucocele)

Case Observations & Histology

- **Clinical Findings (Fig. 8.91):** Raised, blue, moveable, fluid-filled papule/nodule on the left lower lip.
- **Histologic Findings (Fig. 8.92):** Spilled mucin pool surrounded by **granulation tissue** with **no epithelial lining** present.

Q9. The cause of a mucocele (mucus extravasation phenomenon) in Fig. 8.91 would be damage to which of the following?



- a. A major salivary gland
- b. Wharton's duct
- c. The Glands of Blandin-Nuhn
- d. A ruptured salivary accessory gland duct

Correct Answer: D. A ruptured salivary accessory gland duct

- **Clinical Rationale:** Trauma (such as lip biting) severing a minor accessory salivary gland duct causes spilled mucin into surrounding tissues.

Q10. If you were to palpate the area, which of the following would you most likely use as a descriptor?

- a. Very firm nodule
- b. A painful nodule when compressed
- c. The nodule is fixed and non-moveable
- d. A nonpainful moveable fluid-filled nodule

Correct Answer: D. A nonpainful moveable fluid-filled nodule

- **Clinical Rationale:** Mucocoeles characteristically present as soft, fluctuant, non-painful, moveable fluid-filled swellings.

Q11. Which of the following best describes a mucocoele?

- a. Mucous salivary gland secretion spills into the adjacent tissue
- b. A mucocoele is lined with epithelium
- c. A common location for a mucocoele is in the floor of the mouth
- d. Mucocoele is also known as a sialolith

Correct Answer: A. Mucous salivary gland secretion spills into the adjacent tissue

- **Clinical Rationale:** Mucocoeles form when severed minor salivary ducts spill mucin into surrounding connective tissue space.

Q12. A mucocoele contains a surrounding inner tissue. Which of the following lines the mucin pool?

- a. Inflamed epithelium
- b. Multinucleated giant cells
- c. Granulation tissue
- d. Adipose tissue

Correct Answer: C. Granulation tissue

- **Clinical Rationale:** Because mucocoeles are pseudocysts, the mucin pool is walled off by granulation tissue rather than epithelium.

Q13. What is the most common site for a mucocoele?

- a. The tongue
- b. The palate
- c. The lower lip
- d. The upper lip

Correct Answer: C. The lower lip

- **Clinical Rationale:** The lower lip (lateral to the midline) is the most common intraoral site for mucocoeles due to frequent bite trauma.

Q14. Necrotizing Sialometaplasia is due to the necrosis of minor salivary glands. It is believed to be caused by which of the following?

- a. A bacterial component
- b. Vascular ischemia due to malignancy
- c. Trauma to salivary tissue due to mechanical issue such as anesthesia
- d. A viral component affecting the minor salivary glands

Correct Answer: C. Trauma to salivary tissue due to mechanical issue such as anesthesia

- **Clinical Rationale:** Necrotizing sialometaplasia is a benign condition caused by ischemic necrosis of minor salivary glands following local trauma or local anesthetic injections.